Indiana University Health

Work/School Release Form

Employee/Student Information

Name: Reid, Hazel
Address: 2513 W WELLINGTON DR
MUNCIE, IN 473041270
Sex: Female
Date of Birth: 04/16/2018
Phone: (765) 729-0063
Emergency Contact: Reid, Jessica Lynn

Release Details

This notice verifies that the above identified individual was seen in this facility.

Date seen: 02/08/25 22:56:3410:56 pm EST

He/she may return to work/school after this date: 2/10/25

They will return to work with the following restrictions:

__ None
__ No heavy lifting over __ lbs
__ No prolonged standing greater than __ minutes
__ Desk work only
Other:____

Restrictions apply through this date:___

After the date above, they should be able to participate full in all duties and activities.

Provider Name: Carl Pafford, MD Date: 02/08/25 22:56:34 10:56 pm EST

Signature Line
Electronically Signed on 02/08/25 22:57 EST

Pafford, Carl M, MD

Indiana University Health

Reid, Hazel

DOB: 04/16/2018
MRN: 76452910
Visit Date: 02/08/2025

Discharge Instructions

We would like to thank you for allowing us to assist you with your healthcare needs. The following includes patient education materials and information regarding your injury/illness.

Your ED Provider

Time Seen:
Pafford, Carl M, MD / 02/08/2025 21:08

Diagnosis from Today's Visit

Influenza A

What to Do Next

You were treated today on an emergency basis; it may be wise to contact your primary care provider to notify them of your visit today. You may have been referred to your regular doctor or a specialist, please follow up as instructed. If your condition worsens or you can't get in to see the doctor, contact the Emergency Department.

You Need to Schedule the Following Appointments

Follow Up with Westergren, Eric S, DO, FAM
When: Within 5 to 7 days, only if needed

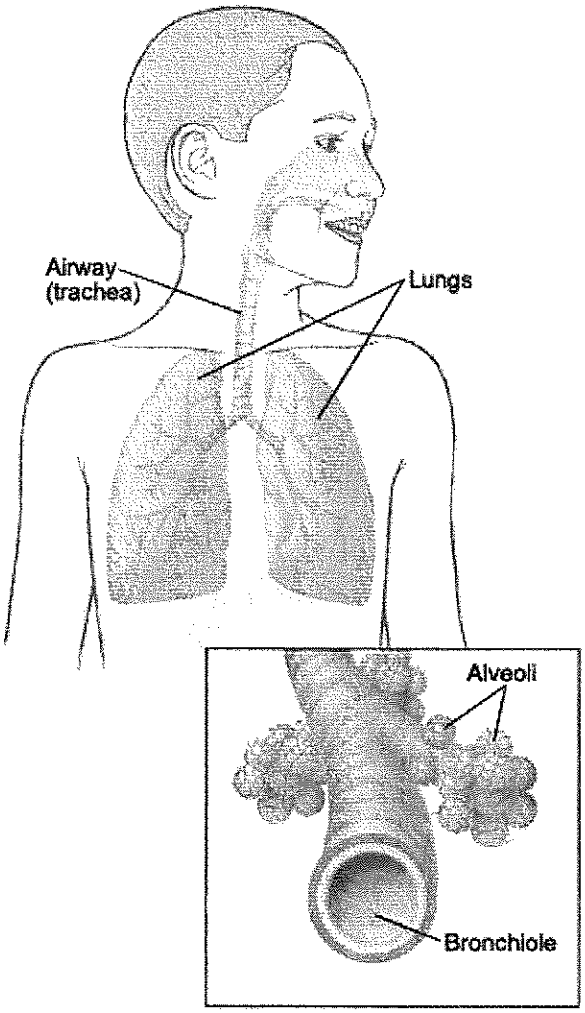
Where: IU Health Ball Memorial Physicians - Yorktown 1420 S Pilgrim Blvd
Yorktown, IN 47396-
(765) 759-4068

Scheduled Follow-Up Appointments

Appointment Type	When	With	Where	Contact Information	Status
Ophth OD Follow Up	03/21/2025 01:15 PM EDT	Graves, April L, OD	OphthP IMR Springmill 10300 N Illinois St Ste 2010 Indianapolis, IN 46290-	(317) 688-3800	Confirmed

Education Materials

Influenza (Child)



Updated for the 2023-2024 flu season

Your child has the flu (influenza). It's a viral illness that affects the air passages of your child's nose, sinuses, throat and lungs. It's different from the common cold. The flu can easily be passed from one child to another. It may be spread through the air by coughing and sneezing. It can also be spread by touching the sick person and then touching your own eyes, nose, or mouth.

Symptoms of the flu may be mild or severe. They can include extreme tiredness (wanting to stay in bed all day), chills, fevers, muscle aches, soreness with eye movement, headache, and a dry, hacking cough.

Your child may be treated with an antiviral medicine if there is risk for or signs of complications. Your child may need antibiotics but only if they have a bacterial infection along with the flu. This might be an ear or sinus infection or pneumonia. Antiviral medicine works best if started within 48 hours of the first symptoms.

Home care

Follow these guidelines when caring for your child at home:

- **Fluids.** Fever increases the amount of water your child loses from their body. For babies younger than 1 year old, keep giving regular feedings (formula or breast). Talk with your child's healthcare provider to find out how much fluid your baby should be getting. If needed, give an oral rehydration solution. You can buy this at the grocery or pharmacy without a prescription. For a child older than 1 year, give them more fluids and continue their normal diet. If your child is dehydrated, give an oral rehydration solution. Go back to your child's normal diet as soon as possible. If your child has diarrhea, don't give juice, flavored gelatin water, soft drinks with caffeine, lemonade, fruit drinks, or frozen ice pops. These may make diarrhea worse.
- **Food.** If your child doesn't want to eat solid foods, it's OK for a few days. Make sure they drink lots of fluid and has a normal amount of urine.
- **Activity.** Keep children with fever at home resting or playing quietly. Encourage them to take naps. Your child may go back to daycare or school when the fever is gone for at least 24 hours. The fever should be gone without giving your child acetaminophen or other medicine to reduce fever. Your child should also be eating well and feeling better.
- **Sleep.** It's normal for your child to be unable to sleep or be irritable if they have the flu. A child who has congestion will sleep best with their head and upper body raised up. Or you can raise the head of the bed frame on a 6-inch block.
- **Cough.** Coughing is a normal part of the flu. You can use a cool mist humidifier at the bedside. Don't give over-the-counter cough and cold medicines to children younger than 6 years of age, unless the provider tells you to do so. These medicines don't help ease symptoms. And they can cause serious side effects, especially in babies younger than 2 years of age. Don't let anyone smoke around your child. Smoke can make the cough worse.
- **Nasal congestion.** Use a rubber bulb syringe to suction the nose of a baby. You may put 2 to 3 drops of saltwater (saline) nose drops in each nostril before suctioning. This will help remove secretions. You can buy saline nose drops without a prescription. You can make the drops yourself by adding 1/4 teaspoon table salt to 1 cup of water.
- **Fever.** Use acetaminophen to control pain, unless another medicine was prescribed. In babies older than 6 months of age, you may use ibuprofen instead of acetaminophen. If your child has chronic liver or kidney disease, talk with your child's provider before using these medicines. Also talk with the provider if your child has ever had a stomach ulcer or digestive bleeding. Don't give aspirin to a child or teen who is ill with a fever. It may cause a serious illness called Reye syndrome. This can affect the brain and the liver.

Follow-up care

Follow up with your child's healthcare provider, or as advised.

When to get medical advice

Call your child's healthcare provider right away if any of these occur:

- Fever (see Fever and children below)
- Fast breathing. If your child is:
    - Age 6 weeks to 2 years, this is more than 45 breaths per minute
    - Age 3 to 6 years, this is more than 35 breaths per minute
    - Age 7 to 10 years, this is more than 30 breaths per minute
    - Older than 10 years, this is more than 25 breaths per minute
- Earache, sinus pain, stiff or painful neck, headache, or repeated diarrhea or vomiting
- Abnormal fussiness, drowsiness, or confusion
- Your child doesn't interact with you as they normally do
- Your child doesn't want to be held
- Your child isn't drinking enough fluid. This may show as no tears when crying, or sunken eyes or dry mouth. It may also be no wet diapers for 8 hours in a baby. Or it may be less pee than normal in older children.
- Rash with fever

Fever and children

Use a digital thermometer to check your child's temperature. Don't use a mercury thermometer. There are different kinds and uses of digital thermometers. They include:

- **Rectal.** For children younger than 3 years, a rectal temperature is the most accurate.
- **Forehead (temporal).** This works for children age 3 months and older. If a child under 3 months old has signs of illness, this can be used for a first pass. The provider may want to confirm with a rectal temperature.
- **Ear (tympanic).** Ear temperatures are accurate after 6 months of age, but not before.
- **Armpit (axillary).** This is the least reliable but may be used for a first pass to check a child of any age with signs of illness. The provider may want to confirm with a rectal temperature.
- **Mouth (oral).** Don't use a thermometer in your child's mouth until they are at least 4 years old.

Use a rectal thermometer with care. Follow the product maker's directions for correct use. Insert it gently. Label it and make sure it's not used in the mouth. It may pass on germs from the stool. If you don't feel OK using a rectal thermometer, ask the healthcare provider what type to use instead. When you talk with any healthcare provider about your child's fever, tell them which type you used.

Below is when to call the healthcare provider if your child has a fever. Your child's healthcare provider may give you different numbers. Follow their instructions.

When to call a healthcare provider about your child's fever

For a baby under 3 months old:

- First, ask your child's healthcare provider how you should take the temperature.
- Rectal or forehead: 100.4°F (38°C) or higher
- Armpit: 99°F (37.2°C) or higher
- A fever of _____as advised by the provider

For a child age 3 months to 36 months (3 years):

- Rectal or forehead: 102°F (38.9°C) or higher
- Ear (only for use over age 6 months): 102°F (38.9°C) or higher
- A fever of _____ as advised by the provider

In these cases:

- Armpit temperature of 103°F (39.4°C) or higher in a child of any age
- Temperature of 104°F (40°C) or higher in a child of any age
- A fever of _____ as advised by the provider

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Allergies

NKA

Additional Information

General Information: We are happy to serve you. The examination and treatment you have received have been on an emergency basis only. Call the Emergency Department for any questions or concerns (listed on front page). An emergency is an illness or injury so serious that you need to get care right away to keep from getting worse. If you do not have an emergency, you should go to your doctor's office. For help finding a doctor, visit www.iuhealth.org and search "Find a Doctor". We are very concerned about your general health and safety. Because of this, when driving or riding in a vehicle, we encourage you to always wear your seat belt, and if you have an infant or child, always use the appropriate infant/child car seat. You were treated today on an emergency basis; it may be wise to contact your primary care provider to notify them of your visit today. You may have been referred to your regular doctor or a specialist, please follow up as instructed.

Medication Instructions: If you need a prescription refill, you must see your doctor. Take all medications only as directed. Do not take a medication you are allergic to. If you have been prescribed new medications check with your pharmacist to make sure there are no problems with the medications you are taking.
To help ensure your safety, keep a copy of this discharge instruction to share with your health care provider at your next visit. It contains a copy of your medication list, diagnoses made in our emergency department and information about treatment you received here.

Referral Instructions: You were treated today on an emergency basis; it may be wise to contact you primary care provider to notify them of your visit today. You may have been referred to your regular doctor or a specialist, please follow up as instructed. If your condition worsens or you can't get in to see the doctor, contact the Emergency Department.

Insurance Information: When you sign in, it is your responsibility to give correct and complete billing information. This is important in case there is a need to contact you with test results. Federal law requires us to provide you a medical screening exam and stabilizing treatment for any emergent medical condition found regardless of your insurance or ability to pay. Your insurance company may require pre-authorization or notification of your visit, both of which are your responsibility. We will make every effort to properly bill your insurance company on your behalf. If you need financial assistance, please call the number listed on front page.

Diagnostic Studies: If you had a diagnostic study performed, we will notify you of any lab, EKG or X-ray findings that would require a change in treatment. A specialist reviews most diagnostic studies after your visit. Some test results are not available for 24-72 hours.

If you have been given medications or a prescription in the Emergency Department: Many medications can make you feel sleepy, impair your ability to drive, or effect your ability to make good decisions. Some of these medicines can even make you stop breathing when you go to sleep, particularly if several different medicines are combined. For this reason, we ask that you arrange for someone else to give you a ride home, do not operate machinery or make important decisions, and do not take any sleeping pills, pain pills, or drink alcohol for at least 4-6 hours from the time you leave the emergency department. It is also important to know that the use of narcotic medications can be habit forming.

Crisis Hotlines:
USA National Suicide Hotline (Toll free and 24/7): Dial 988

Antibiotics Are Not Always the Answer: Antibiotics only treat bacterial infections; they are not needed for viral illnesses. When an antibiotic is not prescribed, ask your healthcare professional for tips on how to relieve symptoms and feel better. For more information on when antibiotics may be helpful, visit www.cdc.gov/getsmart.

Quit Smoking for Health: Smoking has been proven to be hazardous to your health. If you would like to quit smoking, call 1-800-QUIT-NOW (1-800-784-8669) for free counseling and two weeks of free Nicotine Replacement Therapy. Helpful information can also be found online at www.iuhealth.org or the Quitline can also provide information about the closest smoking cessation program to you.

My IU Health: My IU Health allows convenient, electronic access to manage your health. Sign up for a secure account at www.myiuhealth.org.